Newton MUN I Conference
Country: Afghanistan
Delegate: Jules Kitto-Astrop
Committee: World Health Organization (WHO)
Agenda: The Question of Global Health Standards in Vulnerable and Undeveloped Region

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In 2026, national healthcare systems remain a cornerstone of humanities global issues with global staffing shortages and difficulty in access for regions of conflict and vulnerable populations all significantly hindering progress. Afghanistan is a perfect case study to examine the external impacts on healthcare systems, the reason for use or lack thereof, and the numerous challenges faced by the Afghan population in regards to basic health. The delegation of Afghanistan believes in order to provide global health standards that will be attainable for less developed nations, categorizations must be made to ensure the vulnerability is defined through the understanding of status of conflicts, political interference, human rights and violations, religious practices and stepped progress as opposed to working towards one large unattainable goal. Global health standards can simply not be completely standardized but rather guided based on the national context, thus the use of categories will allow the WHO alongside international partnerships to successfully address the most harmful factors and issues while slowly working towards the gold standard attained by developed countries such as; Australia, Canada, and Sweden.

Previously, Afghanistan revealed several concerning statistics regarding healthcare especially for women and children with reports showing only 54% of women had their blood pressure monitored during pregnancy (Rahimzai et al.). Despite these shortfalls, Afghanistan has made great strides in achieving access to care by allowing 90% of the population to access healthcare within 1-2 hours of driving. Although that accomplishment may seem insignificant, Afghanistan sees this as progress towards a better healthcare system that begins with expanding access to rural areas. Furthermore, Afghanistan committed to the creation of national health standards relevant to the context of Afghan populations. We aim to achieve quality through the definition; "a quality health care system is client-centered, equitable, available, appropriate, safe, consistent, effective, timely, and efficient; it continuously improves". Afghanistan believes that though these commitments are strong in theory they lack actionable strategies that can be feasibly implemented in countries like Afghanistan where resources are minimal, staffing is relatively non-existent, and geopolitical issues impact access to and quality of care in all regions.

The delegation of Afghanistan strongly urges all nations to engage in mutually beneficial trade deals regarding healthcare resources and funding in exchange for software use or material exports. Using these international funding partnerships, Afghanistan believes that developing nations with scarce resources and populations could greatly benefit from village health kits including items such as; blankets, first aid kits, non-perishable food items, reusable feminine hygiene products, baby food and formula, clean clothing/covering, a portable washing system, and telemedicine devices connected via satellite to prevent local interference. Moreover, the delegation of Afghanistan firmly believes that the training of locals by the WHO will enable community-based care. Programs should include; basic stitching techniques, CPR, chronic illness and disability care, pregnancy risks, and pre-, neo-, and antenatal care. Crucially, the staffing shortages and "brain drain" in many nations must be addressed, Afghanistan encourages all nations with sustained and stable healthcare systems to provide programs in conjunction with the UNSC to mandate free visas and wages supported by a global fund. Afghanistan also firmly believes that the establishment of a minimum standard for essential medications per person should be calculated to address and resolve inequities. Afghanistan believes patients have the right to peace, thus we strongly urge all nations experiencing or at risk of experiencing conflict to provide legislation on the ban of weapons and on-duty military personnel inside healthcare facilities while also providing a means to request UN volunteers for the securing of healthcare facilities by a neutral body to prevent unnecessary politicization. Finally, Afghanistan believes that the categorization of vulnerability will clarify the need for intervention and extent of foreign aid required. Afghanistan proposes a four tier system where level 1 is described as "at risk of having healthcare interrupted by an external factor" and level 4 is described as "under active attack from a national or international force" which warrants immediate referral to the UNSC under the "Responsibility to Protect" protocols. This provides an actionable plan while prioritizing those most vulnerable and preventing a depletion in global resources.

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